



Medication Management Procedure

Patient's Own Medicines (POM)

Scope

Site	Service/Department/Unit	Disciplines
SCGOPHCG	Hospital-wide	All Medical, Nursing and Pharmacy staff

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Introduction

Patient's Own Medicines (POM) refers to all prescribed and non-prescribed medication brought into hospital by the patient, patient's family, carer or ambulance officers. This includes tablets/capsules, devices (such as inhalers and nasal sprays), patches, injections (such as insulin), eye/ear drops, creams/ointments, suppositories, enemas, liquids and herbal/complementary medicines. POM may be packed in dosage administration aids such as Webster-paks® or dosette boxes. Webster-paks® are prepared by community pharmacies and are labelled with the contents of the pack (medicine, strength, form, etc.). Dosette boxes are plastic containers, usually printed with days of the week and dosing times (e.g. AM, midday, PM, night) that patients/family/carers fill with their own medicines.

Patients should be encouraged to bring their medications to hospital as this has been shown to reduce the occurrence of prescribing errors on admission medication charts.¹⁻³ POM are useful for obtaining a Best Possible Medication History (BPMH) and are beneficial for educating the patient prior to discharge. Where appropriate, POM may also be administered to the patient while they are an inpatient.

POM should be securely stored for the duration of the patient's admission. POM **should not** be sent home with the patient's family/carers as they are useful for obtaining a BPMH and undertaking medication reconciliation, may be required for administration and should be reconciled prior to discharge.

POM may include Schedule 8 (S8) medicines and Schedule 4 Restricted (S4R) medicines. Schedule 8 medicines are also known as Drugs of Dependence and are labelled with 'Controlled Drug' on the original packaging. Schedule 4 Restricted medicines are a group of Schedule 4 drugs which may also be liable to abuse or dependence and are therefore subject to additional storage and recording requirements. S8 and S4R medicines must be securely stored during inpatient admission to reduce risks of diversion and theft and are to be handled and stored according to the legislation and relevant state-wide policy. Voluntary Assisted Dying (VAD) substances are NOT considered as patients' own S8 and S4R medicines.

This procedure outlines the process for the storage, transfer, administration, return and disposal/destruction of POM, including additional requirements for S8 and S4R medicines.

Procedure

1.0 Patient's Own Medicines (POM)

1.1 Storage on Admission

1.1.1 Storage of POM (excluding S4R/S8/fridge items)

- POM are placed in a Patient's Own Medications green plastic zip-seal bag and clearly identified using an addressograph sticker (see Appendix 1). The front of each POM bag in use should be filled out to indicate presence of other POM in the

safe(s) and/or medication(s) in the fridge (i.e. tick the boxes for fridge/S8/S4R and indicate if multiple bags are in use in the 'Bag ___ of ___' section)

- POM should be distinguishable and separated from hospital stock as outlined in the AdPha Standard of Practice in dispensing and distribution for pharmacy services.⁴
- POM may be stored in patients' locked medication cupboards/drawers if space permits or in a designated area in the medication imprest room. Document in the medical record (i.e. the Nursing Admission Assessment form and/or the Medication History and Management Plan) that the patient has brought in their own medicines. Do not send POM to Property for safekeeping.
- Each area should have a system in place to identify patients that have brought in their own medication to ensure they are reviewed on discharge. This may be via placing the green POM bag tear-off receipt or another prompt in the patient's bedside file, a magnet/prompt on the journey board or including in the nursing handover. The Discharge / Transfer Checklist MR637 can also be used to identify the presence of POM and act as a reminder to return these on discharge (for example, by circling relevant boxes in the POMs section).
- At OPH, POM should be stored in the relevant POM baskets in each ward's medication room. Pharmacy staff will return these to Pharmacy for safekeeping until the patient discharges.

Example 1: POM section on SCGOPHCG Discharge / Transfer Checklist Envelope

MEDICATIONS	
Patients Own Medicines (POMs)	Education given to: ☐ Patient ☐ Carer
☐ Returned to patient: ☐ S8 ☐ S4R ☐ Fridge Medicines	Destruction: ☐ POMs consented for Hospital destruction

1.1.2 Storage of personal adrenaline autoinjectors

- An exception to storing medications in a POM bag is for personal adrenaline (epinephrine) auto injectors.
- Refer to section 2.0 'Access to personal adrenaline injector' of [SCGOPHCG Management of Acute Anaphylaxis Guideline](#) for procedure.

1.1.3 Storage of POM that require refrigeration

- Only the item(s) that require refrigeration are stored in a designated space in the medication fridge in a green POM bag.
- The remainder of the medications should be stored in a separate green POM bag as outlined above and both green POM bags filled out to indicate the presence of multiple POM bags (i.e. complete the 'Bag ___ of ___' section).

1.2 Transfer of POM

1.2.1 Transfer between hospital wards/areas

- All POM are to be transferred with the patient when they move between wards/areas. If POM are not transferred with the patient, the receiving nurse should contact the original ward to obtain them (for SCGH ward transfers only).

1.2.2 Inter-hospital transfer

- All POM are to be placed inside the green POM bag for transfer to other sites. The SCGOPHCG Discharge / Transfer Checklist Envelope can be used to indicate the transfer of POMs with the patient. See Example 2 below. Refer to Section 2.2.2 'Inter-hospital transfer' for additional S8 and S4R POM requirements.

- For transfers from Sir Charles Gairdner Hospital to Osborne Park Hospital, green POM bag should be placed inside a red bag with the required medications.

Example 2: Transfer section on SCGOPHCG Discharge / Transfer Checklist Envelope

PATIENTS TRANSFERRED TO ANOTHER FACILITY / HOSPITAL	
☐ Destination notified of transfer plans	☐ Nursing Discharge Summary
☐ Original notes sent with patient to agreed facilities (see ward clerk) or photocopies of relevant medical and nursing notes from current admission	
☐ Infection Control _____	☐ Micro Alerts (specify) _____
☐ Goals of Patient Care form with section 4 complete	
☐ Medicines and POMs transferred with patient [for SCGH/OPH transfers] in red bag placed in green bag.	

1.3 Administration of POM during Admission

- POM can be administered to the patient if:
 - There is a valid prescription on an approved prescription form (e.g. the WA Hospital Medication Chart).
 - The patient provides consent to use their own medication during the admission.
 - The medication is clearly identifiable and in its original packaging. Medications in sealed blister strips or foil outers may only be used if the medication is identifiable.
 - The medication is not expired.
 - The medication has been stored appropriately since being brought into hospital (e.g. items requiring refrigeration have been kept in the fridge from the time that these were brought into the hospital).
- POM should not be administered to the patient if:
 - The medication is not in its original packaging e.g. dosette boxes and Webster-paks®. If a medication is otherwise unavailable, it may be administered from a Webster-pak® if the Webster-pak® is intact, clearly labelled and the medication has been identified using the MIMs Online Product Identification Guide (Pill ID). Consult a clinical pharmacist for advice.
 - The medication is expired. Care should be taken with medications that have specific storage conditions e.g. eye drops are not to be administered if they have been open for more than 28 days, insulin is not to be administered if it has been out of the fridge for more than 28 days and some brands of levothyroxine are not to be administered if stored out of the fridge for more than 14 days. Consult a clinical pharmacist for advice.
 - There is any uncertainty about the identity of the medication, for example when patients transfer medication from its original packaging into another container. The product description on the packaging may not match the contents of the container. Consult a clinical pharmacist for advice.

1.4 Return of POM at Discharge

- POM are patient property and therefore should be returned to the patient at discharge, unless deemed clinically inappropriate by the medical team or pharmacist. If not returned to the patient, steps should be taken to ensure the patient agrees to and is aware of the disposal/destruction.
- Before returning POM at the time of discharge, POM should be reviewed and reconciled with the medication list on discharge. The patient should be counselled about any changes to their existing medication regimen.
- It is the responsibility of the area in possession of the POMs during the admission to facilitate return of the medicines to the patient or to obtain permission for

disposal/destruction. This can be documented on the SCGOPHCG Discharge / Transfer Checklist Envelope (see Example 1 above).

- If POM are not returned on discharge, nursing staff are to contact the patient to inform them that POM have been left behind and provide the opportunity to collect their POM. A family member/carer/agent may collect medicines on the patient's behalf. Contact may be documented using Appendix 2: Communication sheet for Patient's Own Medicines left on a ward'.
- After discharge, if the patient cannot be contacted, does not wish to collect their POM and consent for its destruction, or they have not collected their POM by the end of the agreed timeframe the medicines are to be disposed of or destroyed.

1.5 Disposal/destruction of POM

- POMs can only be disposed/destroyed, with consent, if there is a medication safety risk (e.g. multiple strengths of same medicine), the medicine is no longer required, the medicine is expired, or if the patient requests.
- If it is not clinically appropriate to return the medicine to the patient, steps should be taken to ensure the patient or their next of kin/carer agree to and are aware of the disposal. The decision should be documented in the patient's medical record.
- POM can only be returned to pharmacy for destruction in the above circumstances, if they have not collected their POM by the end of the agreed timeframe, if the patient is unable to be contacted after several unsuccessful attempts, or if the patient is deceased.
- At SCGH, POM are disposed of in the yellow clinical waste bins on the ward. Notify pharmacist of S8/S4R and refrigerated POM for collection – DO NOT place in the return to pharmacy box.
- Consent for disposal/destruction at the time of discharge can be documented on the SCGOPHCG Discharge / Transfer Checklist Envelope.
- If the patient is contacted (including unsuccessful attempts) after discharge and the POM are to be disposed/destroyed, this may be documented using Appendix 2: Communication sheet for Patient's Own Medicines left on a ward' or in the S8/S4R POM register. This may be useful in the event of future queries.

2.0 Additional requirements for S8 and S4R POM

2.1 Storage of S8 and S4R POM on Admission

- A chain of custody must be maintained for S8 and S4R POM.
- Patient's own S8s and S4R medicines must be entered into the relevant S8 or S4R POM register and stored in a designated space in the appropriate safe.
- Where clinically appropriate and possible, the physical count should be undertaken in front of the patient and the patient should agree with the result of the physical count.
- Patient's own S8s and S4Rs may be stored and recorded either by:
 - Sealing inside an S8 or S4R security bag
 - When this is not feasible, storing outside of a security bag and entering each medication on a separate page of the register.
- If the patient has other medicines in a green POM bag, tick the S8/S4R medication box on the bag
- If the patient has S8 or S4R medications within a Webster-Pak® the entire Webster-Pak® must be stored in the S4R or S8 safe respectively. If a Webster-Pak® contains both S4R and S8s it should be stored in the S8 safe. The same applies for other dose administration aids such as dosette boxes.

- If at any stage a medicine discrepancy is identified, it must be reported in accordance with hospital policy using the Schedule 8 and restricted Schedule 4 Medicine discrepancy/loss report form.

2.1.1 Security Bags for Patient's Own S8 and S4R Medicines

- Specifically designed plastic security bags have a unique ten-digit serial number and tamper-evident seals.
- Once item(s) are placed into the bag and the contents of the bag are documented on a piece of paper called an insertion slip (see example 3), the bag (including the insertion slip) is sealed. Daily checks will then be undertaken to ensure the integrity of the bag (i.e. that the seal is intact) and removes the need to count each individual item. At OPH, the sealed bag should be returned to Pharmacy with an S4R/S8 requisition slip noting the medication, patient name and ten-digit serial number of the bag to be stored in the Pharmacy safe.
- The use of security bags should therefore reduce the amount of time required to handle and store patient's own S8 and S4R medicines that are brought into hospital.

2.1.2 Process of entering S8 and S4R POM into registers using security bags

- Two authorised staff members (doctor, nurse practitioner, registered nurse, medicines competent enrolled nurse, pharmacist; one of whom must be a registered nurse) are required to sign medicines and security bags into the register.
- Use the relevant S8 or S4R security bag according to the schedule of the medicine(s).
- Do not place S8 and S4R medicines into the same bag. The only exception is for Webster-pak® or other dose administration aids which contain both S8 and S4R medications – in this case a S8 security bag is used.
- The front of the security bag has an opaque section "Description of Contents" (see Appendix 3: S8 and S4R Security Bags). This is NOT to be used.
- Complete the 'Insertion Slip for Patient's Own S8 Medication' or the 'Insertion Slip for Patient's Own S4R Medication' as appropriate (see Appendix 4). Only POM which are clearly identifiable are to be recorded. Loose tablets or capsules which cannot be identified should not be recorded – seek advice from a clinical pharmacist if necessary.
- An insertion slip is required for each security bag.
- Complete all details clearly and accurately. Use the slip to document whether there are other S8 or S4R security bags in use, whether there are S8 or S4R medicine(s) not being stored in security bag(s) (e.g. if being used for administration during admission) and whether there are non-S8/S4R POM (stored in green POM bag). The second box on the slip is for the patient's addressograph label.
- Place the insertion slip facing OUTWARDS to the back of the bag so that the insertion slip details can be read when the bag is sealed. Note that the front of the bag has both wording and an opaque section which may obscure the details of the insertion slip should the insertion slip be facing the front of the bag.

Example 3: Sample insertion slip

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Insertion Slip for Patient's Own **S8** MedicinesIn **addition** to this bag there are:

1 total number of S8 security bags in use
0 no. of S8 medications not in security bag
1 total number of S4R security bags in use
0 no. of S4R medications not in security bag
 bag [] non-S4R/S8 medications (green bag)

Patient Label

 Ward G71 Security Bag Serial Number: ABC1234567

Drug/medication name	Strength	Form/Preparation	Quantity
Oxycodone (Endone)	5mg	IR tablet	5
Oxycodone/naloxone (Targin)	20/10mg	SR tablet	10

 Name (print): Nurse One Signature: Nurse One Designation: RN

 Name (print): Nurse Two Signature: Nurse One Designation: RN

 Date: 11/07/2022 Time: 14:30

- Affix a patient addressograph label to the security bag in the designated position on the top left corner of the bag and seal the bag by removing the foil strip to reveal the adhesive surface.
- The patient's medication receipt on the top of the bag will NOT be issued to the patient – this piece can be detached and used as a prompt to remind staff to return POM on discharge (e.g. attach this to the green POM bag tear-off receipt, place in bedside file or attach to the discharge envelope in preparation for discharge).
- Enter the details of the POM into the appropriate register (see Appendix 5 and 6).
- Use one page of the register per security bag. If the patient's medications cannot fit into one bag and/or there are additional medications after the first bag has been sealed, use another separate security bag and page in the register.
- Document the patient's name, the serial number of the security bag and the items in the bag (i.e. those listed on the insertion slip) in the register. It is important that the details of the insertion slip and the register page match. Should the security bag be misplaced (along with the insertion slip) this may be the only place where details of the POM are documented.
- Record the patient name on the index page of the register (see Example 4).

Example 4: Sample index page of register

Patient Jane Smith has one security bag of POM. Patient Joe Campbell has two bags in total. The register page for patient Zoe Wells is closed/inactive (medications returned to patient on discharge or transferred to another hospital ward/area).

Note that crossing out a patient's name on the index page is acceptable and makes it easier to see how many bags are still currently in the register and the safe. Cross out neatly and do not obliterate details. Whilst it is acceptable to cross out on the index page, if errors are made on register pages, do not cross out but rather make amendments on the next line or in the margins of the page

Drug (Generic Name)	Brand Name	Release Type	Form	Strength	Page
Jane Smith					<u>1</u>
Joe Campbell	(Bag 1 of 2)				<u>2</u>

Joe Campbell	(Bag 2 of 2)				3
Zoe Wells					4

2.1.3 Process of entering S8 and S4R POM into registers when not using security bags

- Tamper evident security bags should be used wherever possible to store S8 and S4R POM. However, it may be appropriate NOT to use the security bags in certain situations, for example when the patient is charted for a medication which is not available in the hospital (after hours or within hours) and the POM is available and suitable for administration. This is not recommended when the POM is an S8 or S4R oral liquid (see section 2.3 below).
- If security bags are not used, enter each individual medication onto a separate page of the appropriate register and perform daily checks (ensuring the actual count matches the balance in the register).

2.1.4 Patient's own S8 and S4R medicines in dose administration aids

- If POM is in the form of a Webster-pak® or dosette box contains S8/S4R medications, the entire pack should be placed into the security bag (i.e. do not break open the pack).
- When entering medications within Webster-paks® into the registers, staff are documenting the contents of the pack based on the written description of the pack contents and how many doses remain intact based on visual inspection of the pack (e.g. tramadol stated to be packed as one 50mg tablet twice daily, 10 of 14 dose times are intact/sealed/unbroken, therefore a quantity of 20 tablets is entered into the register).
- Where a Webster-pak® or dosette box contains both S8 and S4R medications, use the 'Insertion Slip for Patient's Own S8 Medication' to record both S8 and S4R medications, enter the details of the security bag into the S8 POM register and store as a S8 POM.
- Where possible, use the larger sized security bag. The extra space inside the bag may minimise the obscuring of details on the Webster-pak® (i.e. allow inspection of details on the pack without having to open the bag).

2.1.5 Patient's own S8 and S4R liquid medications

- Security bags should be used when storing patient's own S8 and S4R liquid medicines, regardless of whether they are used for administration. This is to reduce the chance of tampering and diversion of stock during the time it is stored in the safe (when received from the patient until it is either returned to the patient, or transferred to pharmacy for destruction). Due to the potential for volume loss with viscous liquids, any oral liquids should not be measured upon receipt.
- Upon receipt of the medicine, a piece of writable tape is to be applied to the side of the bottle and the current liquid level marked. An estimate of the contained volume may be made with a visual inspection by the nursing staff and with agreement of the patient. This agreed volume is to be documented on the security bag insertion slip as the received volume.
- Inventory checks of S8 and S4R liquid medications are not to involve transfer, decanting or physical measurement, as this can result in further measurement loss, reduced potency, altered shelf life and potential contamination.
- If a visual inspection identifies a suspected discrepancy at any stage, then the standard process for reporting discrepancies must be followed.

2.1.6 Daily management of S8 and S4R POM

- Daily checks are required for Patient's Own S8 and S4R medications stored in clinical areas.
- Two staff (one of whom must be an RN) are required for the daily checks.
- Staff are required to perform an integrity check to confirm that the tamper-evident security bags are intact. Match the security bag serial number on the bag to the number documented at the beginning of each page (the first entry should include this security bag serial number). There is no requirement to confirm that the contents of the bag are consistent with the insertion slip details if the seal is intact.
- If the bag is intact, document "Bag intact" (see Appendix 5).
- The balance will be the number of items in the bag as stated on the insertion slip.
- If the bag seal is not intact or the bag is missing, follow NMHS Discrepancy Management [protocol](#) for reporting loss of S8 and S4R medications.
- Daily checks can be signed for at the back of the register, however at least once per week the daily check should be documented on each active page of the register.

2.2 Transfer of S8 and S4R POM

2.2.1 Transfer between hospital wards/areas

- All Patient's Own S8 and S4R medications, whether a security bag is in use or not, need to be transferred physically between wards/areas by one authorised staff member (doctor, nurse practitioner, registered nurse, medicines competent enrolled nurse, pharmacist).
- This does not include patients who are being discharged (e.g. to/via the Discharge Ward).
- When transferring S8 or S4R POM between ward areas, the whole bag is transferred, without opening it.
- Documentation of transfers (out of one register and into another register) require the signature of two authorised members of staff (one of whom must be an RN) in both registers
- "To ____ (ward/area)" to be entered into the register when transferring stock. Balance (number of items in the bag) would then read "NIL" (see Appendix 5). The page should then be closed and also removed from the index page by drawing a line through the patient name (to aid in daily checks as it makes it easier to see which pages are still active/in use – see Example 4).
- "From ____ (ward/area)" to be entered into the register of the ward/area receiving stock (see Appendix 6). Enter details of items in bag and balance into the register based on the details of the insertion slip enclosed in bag.

2.2.2 Inter-hospital transfer

- For interhospital transfers, S8 and S4R POM should be placed into the green POM bag for transfer with the patient and the sealed security bag should remain intact. At destination site, staff are expected to confirm tamper evident bag is intact.
- Two approved staff members must sign the S8/S4R POM out of the register, documenting the hospital for transfer. An authorised staff member must communicate with the receiving site that S8/S4R POMs are being transferred with the patient and document in the healthcare record (e.g. SCGOPHCG Discharge / Transfer Checklist Envelope).

2.3 Administration of S8 and S4R POM during Admission

- In some circumstances (e.g. medication not available in the hospital), it may be appropriate to administer S8 and S4R POM.
- Administration of liquid oral S8/S4R POM is associated with risks, for example, an end of bottle reconciliation cannot be performed when the initial volume is unknown. For this reason, it is preferred to avoid use of liquid oral S4R/S8 POM for dosing during admission wherever possible.
- If this is unavoidable and the oral liquid S8/S4R POM is deemed appropriate for administration during the admission, an incremental volume scale should be employed on the bottle label and the remaining volume level should be recorded after each required dose is withdrawn.
- If a dose is needed, measure out the required dose and seal in a new bag. This process is to be repeated for every dose dispensed from the liquid S8/S4R POM. This reduces diversion risk during time periods between administered doses.

2.4 Return of S8 and S4R POM at Discharge

- Requires the signature of two authorised members of staff (doctor, nurse practitioner, registered nurse, medicines competent enrolled nurse, pharmacist; one of whom must be a registered nurse)
- 'Returned to patient on discharge' to be entered into the register. Balance (number of items in the bag) would then read "NIL". The page should then be closed and also removed from the index page by drawing a line through the patient name (to aid in daily checks as it makes it easier to see which pages are still active/in use – see Example 4).
- Return sealed bag to patient. Do not open the bag (see Appendix 6).
- If return is considered inappropriate or unsuitable, staff must inform the patient or next of kin/carer, however consent to dispose or destroy may not be required.

2.5 Disposal/destruction of S8 and S4R POM

- Consent for disposal/destruction of S8/S4R POMs should be documented, either on the relevant page in the corresponding S8 or S4R register, or in the patient's medical record (e.g. SCGOPHCG Discharge / Transfer Checklist Envelope or patient's medical record)..
- When destruction is required, complete a requisition form and send to inpatient pharmacy. S8/S4R POMs must be signed out of the register by two approved staff and a requisition slip used to facilitate destruction in pharmacy.

3.0 Practical Points

3.1 Ordering of POM bags

- POM bags are stocked on all wards and purchased by the Nurse Manager

IPROC Ordering Number	Item Description	Measurements	Supplier Item
201574G	Bag,Plastic:Flat;"Patient's Own Medication Bag";With Tear Off Receipt;Green/White;430mm Deep X 350mm + 80mm Receipt;Bag Of ;Patients Name/UMRN;Date Discharged Table;Other POM;Resealable Bag	43cm x 35cm x 8cm	MS 100

3.2 Supply of security bags and insertion slips

- Security bags are purchased by the Nurse Manager. There are two types, one specific for S4Rs and one specific for S8s and there are two sizes of each bag available (small and large). Small bags are approximately A5 in size and large bags are approximately A4 in size. See Appendix 3: S8 and S4R Security Bags.
- Insertion slips for documenting the details of the contents of the security bag are located on the SCGH Pharmacy page of the HealthPoint SCGOPHCG intranet site in the 'Controlled Drug Management' section ('Insertion Slip for Patient's Own S8 and S4R Medicines'). See Appendix 4: Insertion Slips. The insertion slip document is two-to-a-page with the S8 slip being on the first page and the S4R slip on the second page. For convenience, these slips may be pre-printed and stored with yet-to-be-used security bags or printed as required.
- Ordering details of security bags:

IPROC Ordering Number	Item Description	Measurements	Supplier Item
154412A	Bag, Plastic: Flat; "Patient's Own Medication"; S4R Large; Tamper Evident; Tear Off Receipt; Barcode & Sequential Number; 75 Micron Co-Ex Film	26x34cm	BAG_S4D/S4R340
154413B	Bag, Plastic: Flat; "Patient's Own Medication"; S4R Small; Tamper Evident; Tear Off Receipt; Barcode & Sequential Number; 75 Micron Co-Ex Film	19x26cm	BAG_S4D/S4R260
154411Y	Bag, Plastic: Flat; "Patient's Own Medication"; S8 Large; Tamper Evident; Tear Off Receipt; Barcode & Sequential Number; 75 Micron Co-Ex Film	26x34cm	BAG_S8340
154414C	Bag, Plastic: Flat; "Patient's Own Medication"; S8 Small; Tamper Evident; Tear Off Receipt; Barcode & Sequential Number; 75 Micron Co-Ex Film	19x26cm	BAG_S8260

Related Documents

SCGOPHCG Medicines Management Policy and Procedure

SCGH Nursing Practice Guidelines Medication Management

SCGH Guide for Medication cleaning process following exposure to confirmed or suspected COVID-19

WA Health Mandatory Policy Medicines Handling Policy MP0139/20

WA Health Mandatory Policy Reporting of Schedule 4 Restricted and Schedule 8 Medicines Discrepancies Policy MP 0103/19

WA Health Guideline on oral liquid Schedule 4 Restricted and Schedule 8 medicines

WA Health Guideline on patients own medicine

WA Health Risk based requirements for medicines handling

WA Health Guidelines for dealing with a Schedule 4 Restricted and Schedule 8 Medicines Discrepancy

NMHS Administration of Schedule 8 (S8) and Schedule 4 (S4R) Medicines Policy and Procedure

NMHS Destruction, Discard and Disposal of Schedule 8 (S8) and Schedule 4 Restricted (S4R) Medicines Policy

NMHS Destruction, Discard and Disposal of Schedule 8 (S8) and Schedule 4 Restricted (S4R) Medicines Procedure

NMHS Discrepancy Management of Schedule 8 (S8) and Schedule 4 Restricted (S4R) Medicines Policy

NMHS Inventory of Schedule 8 (S8) and Schedule 4 Restricted (S4R) Medicines

NMHS Management of Patients' Own Schedule 8 (S8) and Schedule 4 Restricted (S4R) Medicines Policy
 NMHS Transport of Schedule 8 (S8) and Schedule 4 Restricted (S4R) Medicines Policy
 Medicines and Poisons Act 2014
 Medicines and Poisons Regulations 2016

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3. Lummis H, Sketris I and Veldhuyzen van Zanten S. Systematic Review of the use of patient's own medications in acute care institutions. *J Clin Pharm Ther* 2006;31(6).
4. Society of Hospital Pharmacists of Australia. Standard of Practice in dispensing and distribution for pharmacy services. *J Pharm Prac Res* 2021;51(6):511-535.

Appendices

Appendix 1 – Patient's Own Medications green plastic zip-seal bag
 Appendix 2 – Communication sheet for Patient's Own Medicines left on a ward
 Appendix 3 – S8 and S4R Security Bags (note that there are two sizes available)
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Authorisation & Version Control

Policy Sponsor	Director of Clinical Services				
Policy Contact	SCGOPHCG Medicines and Therapeutics Committee Executive Officer (Senior Pharmacist)				
Date First Issued:	10/08/2023	Last Reviewed:	11/08/2025	Review Date:	30/08/2028
Version No.	3.2	Reference #:	MMP003		
Approved by:	SCGOPHCG Medicines and Therapeutics Committee			Date:	11/08/2025
Acknowledgments:	Author: Michelle Sweeney (Medication Safety Pharmacist, Pharmacy)				
Standards Applicable	Insert relevant NSQHS Standards (Version 2): 				

For full details regarding, development, review, consultation, approval, endorsement - contact the Policy Officer for the submission form

This document can be made available in alternative formats on request for a person with a disability.

Appendix 1: Patient's Own Medications green plastic zip-seal bag

PATIENT'S OWN MEDICATIONS stored on ward.
Ensure reconciliation & return to patient on discharge.

Storage Location 1: _____
Storage Location 2: _____
Storage Location 3: _____

Hospital: _____
Ward: _____

Patient addressograph

Bag _____ of _____

PATIENT'S OWN MEDICATIONS:

PATIENT'S NAME/UMRN
(ADDRESSOGRAPH)

- Keep in patient's locked medication drawer or secure location
- Return to patient on discharge if appropriate

Enter POM:

S4R ☐

S8 ☐

Fridge ☐

Date (discharged)	/ /
Telephone Number	
Phone call to pick up	Y / N
Will collect on	/ /
For destruction	Y / N

Appendix 2 – Communication sheet for Patient's Own Medicines left on a ward (applicable to SCGH only)

Please note, once medications have been taken to pharmacy for destruction, they cannot be returned.

Patient's name	Contact number	Date and time patient was contacted	Patient to collect POM (Y/N)	Date/time patient will collect POM	Patient consent for POM to be destroyed (Y/N)	Contacting nurse (sign)	Contacting nurse (legibly print name)	Dispose of POM in ward yellow clinical waste bin (non S8/S4R), or request return to pharmacy for destruction of S8 and S4R POM (dispose/request return to pharmacy)

Appendix 3 – S8 and S4R Security Bags (note that there are two sizes available)

PATIENT'S S8 MEDICATION RECEIPT

PATIENT'S LABEL

Date: / /

AAE0015998

PRE0005306

PATIENT'S S8 MEDICATION BAG

PATIENT'S LABEL

AAE0015998

Description of Contents:

S8

www.tamperevident.com.au

PRE0005306

PATIENT'S S4R MEDICATION RECEIPT

PATIENT'S LABEL

Date: / /

AAD0010497

PRE0005306

PATIENT'S S4R MEDICATION BAG

PATIENT'S LABEL

AAD0010497

Description of Contents:

S4R

www.tamperevident.com.au

PRE0005306

Appendix 4 – Insertion slips (applicable to SCGH site only)

	Insertion Slip for Patient's Own S8 Medicines	<i>Patient Label</i>																																					
In addition to this bag there are: ___ total number of S8 security bags in use ___ no. of S8 medications not in security bag ___ total number of S4R security bags in use ___ no. of S4R medications not in security bag ☐ non-S4R/S8 medications (green bag)																																							
Ward: _____ Security Bag Serial Number: _____																																							
<table border="1" style="width: 100%; border-collapse: collapse;"> <thead> <tr> <th style="width: 33%;">Drug/medication name</th> <th style="width: 15%;">Strength</th> <th style="width: 33%;">Form/Preparation</th> <th style="width: 19%;">Quantity</th> </tr> </thead> <tbody> <tr><td> </td><td> </td><td> </td><td> </td></tr> <tr><td> </td><td> </td><td> </td><td> </td></tr> <tr><td> </td><td> </td><td> </td><td> </td></tr> <tr><td> </td><td> </td><td> </td><td> </td></tr> <tr><td> </td><td> </td><td> </td><td> </td></tr> <tr><td> </td><td> </td><td> </td><td> </td></tr> <tr><td> </td><td> </td><td> </td><td> </td></tr> <tr><td> </td><td> </td><td> </td><td> </td></tr> </tbody> </table>	Drug/medication name	Strength	Form/Preparation	Quantity																																			
Drug/medication name	Strength	Form/Preparation	Quantity																																				
Name (print): _____ Signature: _____ Designation: _____ Name (print): _____ Signature: _____ Designation: _____ Date: _____ Time: _____																																							

	Insertion Slip for Patient's Own S4R Medicines	<i>Patient Label</i>																																					
In addition to this bag there are: ___ total number of S4R security bags in use ___ no. of S4R medications not in security bag ___ total number of S8 security bags in use ___ no. of S8 medications not in security bag ☐ non-S4R/S8 medications (green bag)																																							
Ward: _____ Security Bag Serial Number: _____																																							
<table border="1" style="width: 100%; border-collapse: collapse;"> <thead> <tr> <th style="width: 33%;">Drug/medication name</th> <th style="width: 15%;">Strength</th> <th style="width: 33%;">Form/Preparation</th> <th style="width: 19%;">Quantity</th> </tr> </thead> <tbody> <tr><td> </td><td> </td><td> </td><td> </td></tr> <tr><td> </td><td> </td><td> </td><td> </td></tr> <tr><td> </td><td> </td><td> </td><td> </td></tr> <tr><td> </td><td> </td><td> </td><td> </td></tr> <tr><td> </td><td> </td><td> </td><td> </td></tr> <tr><td> </td><td> </td><td> </td><td> </td></tr> <tr><td> </td><td> </td><td> </td><td> </td></tr> <tr><td> </td><td> </td><td> </td><td> </td></tr> </tbody> </table>	Drug/medication name	Strength	Form/Preparation	Quantity																																			
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Name (print): _____ Signature: _____ Designation: _____ Name (print): _____ Signature: _____ Designation: _____ Date: _____ Time: _____																																							

Appendix 5 – Register entry for recording S4R POM receipt in a security bag, daily check and transfer to another ward/area

Register entry – example of POM being received from the patient, a security bag being used to seal in received POM, security bag checked (daily check to occur on each page at least once weekly with remainder of daily checks documented at the back of the register) and security bag being transferred to another ward/area. Transfer the intact bag – do NOT open the bag to verify the contents prior to transferring bag.

RECORD OF DRUGS: RECEIVED, ADMINISTERED OR SUPPLIED

DRUG (including strength & form) Jane Smith Balance: nil Brought forward from page: new entry

Date	Received		Identifying details	Administered or Supplied							
	Amount	Requisition number	Full name of patient or supplier	Time	Dose administered	Amount discarded	Amount issued from stock	Balance	Name of Prescriber	Administered, Supplied or Received By (sign& print name)	Witnessed by (sign & print name)
01/09/2022	5	Serial Number ABC1234567	From Patient Temazepam 10mg tab	0945				5		Nurse ONE Nurse ONE	Nurse TWO Nurse TWO
	3		From Patient Tramadol 50mg IR cap	0945				3		Nurse ONE Nurse ONE	Nurse TWO Nurse TWO
02/09/2022			Bag intact	1045				2 items		Nurse ONE Nurse ONE	Nurse TWO Nurse TWO
03/09/2022		12345	Transferred to ward G64	1115			2 items	NIL		Nurse ONE Nurse ONE	Nurse TWO Nurse TWO

*** CLOSE PAGE e.g. by drawing a diagonal line through the remaining space on the page and cross out the patient's name on the index page ***

Appendix 6 – Register entry for receipt of transfer from another ward/area and return to patient on discharge

Register entry – example of intact security bag being received from another ward/area and subsequently being returned to the patient on discharge. Upon receipt from another ward/area, record the serial number and the number of items in the bag according to insertion slip. Do NOT open the bag to verify the contents

RECORD OF DRUGS: RECEIVED, ADMINISTERED OR SUPPLIED

DRUG (including strength & form): Jane Smith Balance: nil Brought forward from page: new entry

Date	Received		Identifying details	Administered or Supplied							
	Amount	Requisition number	Full name of patient or supplier	Time	Dose administered	Amount discarded	Amount issued from stock	Balance	Name of Prescriber	Administered, Supplied or Received By (sign& print name)	Witnessed by (sign & print name)
04/09/2022	2 items	Serial Number ABC1234567	From ED Temazepam 10mg	1200				5		Nurse ONE Nurse ONE	Nurse TWO Nurse TWO
04/09/2022			From ED Tramadol 50mg IR caps	1200				3		Nurse ONE Nurse ONE	Nurse TWO Nurse TWO
05/09/2022			Bag intact	1035				2 items		Nurse ONE Nurse ONE	Nurse TWO Nurse TWO
07/09/2022			Returned to patient on discharge	1020			2 items	NIL		Nurse ONE Nurse ONE	Nurse TWO Nurse TWO

*** CLOSE PAGE e.g. by drawing a diagonal line through the remaining space on the page and cross out the patient's name on the index page ***